



Triage Assessment of Referrals in Community Mental Health

Policy code	SMHS:14
Original endorsement date	01 January 2003
Date of current endorsement	02 February 2024
Functional subgroup	Mental Health
Summary	South Metropolitan Health Service (SMHS) will provide timely and equitable access to a community mental health service through an effective triage process conducted by mental health clinicians. All person's self-presenting at a SMHS mental health service will be initially triaged by the Mental Health Triage Officer (or their equivalent) who will assess, formulate and plan appropriate interventions.
Policy owner	Executive Director Clinical Service Planning and Population Health
Contact	Health Service Planner, Anureet Johl, 6152 1169
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NSQHS Standards	Standard 1: Clinical Governance Standard 2: Partnering with Consumers Standard 5: Comprehensive Care Standard 6: Communicating for safety
Chief Psychiatrists Standards for Clinical Care	Assessment Risk assessment and management
Status	Active

Note: The HealthPoint Information Hub links in this policy are accessible only from WA Health computers

1. Background

1.1. About this document

SMHS will provide timely and equitable access to a community mental health service through a triage process conducted by appropriately qualified mental health clinicians. All person's self-presenting at a SMHS mental health service will be initially triaged by the Mental Health Triage Officer (or their equivalent) who will assess, formulate and plan appropriate interventions in consultation with the Multidisciplinary Team (MDT).

1.2. Key definitions

Term	Definition
Assessment	The systematic evaluation of information about a person in order to ascertain their diagnosis, needs and desired outcomes of treatment. Biopsychosocial assessment may be ongoing and forms the basis for the development and review of an individual care plan in collaboration with the person, their family, carers and significant others.
Intake	The Multi-disciplinary discussion of a referral occurring at the next scheduled Intake meeting after receipt. The MDT will determine the level of input and/or treatment required for all referrals.
Referral	A request to access services for specialised mental health interventions from an individual, community and non-government agency or any other health service provider.
Triage	The process of determining the urgency of response for a consumer, based on a referral or presentation to a mental health service.
Duty of care	The legal obligation of healthcare professionals to patients to adhere to a standard of reasonable care.
A – Extreme urgency	Immediate response/intervention due to clear and imminent serious risk of harm to self or others. Very high unpredictability disorganisation, acute disturbance. May include police / ambulance notification.
B – High urgency response	(Contact within 2 hours, where possible) Due to serious risk to self or others. High unpredictability, disorganisation, acute disturbance.
C – Medium urgency	(Contact within 12 hours, where possible) Due to harm, crisis, distress benefit of early intervention.
D – Low urgency	(Contact within 48 hours, where possible) Due to client / carer distress, early symptoms of psychosis, person known to the service requiring priority review.
E - Non-Urgent	(See within 2 weeks) A routine referral is for a non-urgent assessment which will be managed at the next intake meeting.

2. Policy Statement

This policy describes the Triage assessment process for prioritisation of new referrals to South Metropolitan Health Service (SMHS) mental health services. Refer to the Department of Health 'Triage to Discharge' Mental Health Framework for State-wide Standardised Clinical Documentation.

3. Roles and Responsibilities

3.1. Executive

Each SMHS site Service Director/ Co-Director will ensure that managers and supervisors are aware of the requirement to implement guidelines and safe work procedures relating to Triage assessment of referrals in community mental health.

3.2. Managers/Supervisors

Managers and supervisors should:

- Undertake appropriate risk assessments of tasks involved in Triage assessment of referrals
- Ensure the development of relevant local protocols and procedures, if required
- Ensure staff are trained in relevant safe work procedures and manual tasks
- Ensure appropriate training and education is provided to staff in relation to Triage assessment of referrals in community mental health.

3.3. Staff

Staff should:

- Comply with mandatory training requirements and proactively seek further training as required
- Ask for assistance if required and involve relevant medical staff in the decision-making process regarding Triage assessment of referrals in community mental health.
- Comply with safe work procedures

4. Referrals

Written, verbal or e-Referrals may originate from sources such as:

- General Practitioners (GP);
- Emergency Departments;
- Another mental health service;
- Community agencies;
- Western Australia Police;
- Personal presentation/self-referral / Families;
- Schools.

For written referrals and persons self-presenting to a mental health service Assessment and Treatment Team (ATT) seeking access to services for themselves, with or without a referral, clinical staff will:

- Assess the appropriateness of the referral
- Gather additional information from other sources including:
 - consumer
 - medical record
 - webPSOLIS and PSOLIS (including Alerts)
 - previous discharge summary
 - other mental health services
 - carers/family / significant other(s)
 - General practitioner
- Determine the level of risk and urgency of response required.

If the person requires immediate medical attention, refer to the nearest Emergency Department and/or call an ambulance.

Refer to the [SMHS Acute Response Mental Health Emergencies/Crises Policy](#).

All referrals will be referred to the intake process for the service / program, taking into account the assessed level of risk and degree of urgency, which will influence the timing of the team's response.

The intake process consists of a multidisciplinary team meeting where decisions are made regarding the appropriateness of the referral and determine the outcome of the referral i.e. further triage, or acceptance into the team and allocation of care coordinator, or recommendations to referrer if not accepted into the team which may include facilitation or referrals to external agencies if required, as well as updating PSOLIS, if no further action is required.

If the referral is considered inappropriate for tertiary mental health care, post MDT discussion intake, clinicians will provide feedback to the referrer and if required facilitate a referral to a relevant service.

5. Triage

Persons conducting triage assessment must be experienced senior mental health professionals or under the supervision of a senior mental health professional. Clinical staff will initially assess the appropriateness of the referral / self-referral using their local mental health service admission criteria. Staff will determine the level of risk and urgency by interviewing the person, and discuss the completion of the SSCD Triage form risk assessment and gather further information from other sources including:

- Medical record
- Discharge summary
- webPSOLIS and PSOLIS
- Other mental health services

Staff will also seek information from family / carers / significant others with the consumer's consent wherever possible unless otherwise permitted or required by law. Triage should be respectful of the rights of carers and consumers and is an opportunity to identify the cultural or linguistic requirements of the consumer.

- Refer to the Carers Recognition Act 2004.
- Refer to Department of Health (DOH) WA Health Consent to Treatment Policy

The triage process includes undertaking an initial assessment that identifies the need for access to mental health services. This initial assessment information will identify the urgency of need and timeframe the person should be reviewed. If risk is clear and imminent emergency action should be taken.

Refer to the [SMHS Care Coordination in Mental Health Policy](#) Triage Assessment.

Once the clinician has completed an initial triage assessment which lists the patients current identified problems with psychiatric history and mental state examination a risk assessment and management plan is to be initiated and discussed at MDT unless risk is clear and imminent.

Where it is considered that the person is at clear and immediate risk, the person is to be triaged as a Category A – immediate response. Referrals assessed as requiring an immediate response must be seen face-to-face by the Assessment and Treatment Team (ATT) clinician(s). If despite every effort the ATT are unable to visit, or it is deemed unsafe to visit, the WA Police assistance should be requested.

Where the person's presentation is unclear, the decision regarding the next steps should be will be undertaken following a detailed medical assessment in discussion with the MDT.

Once the person has been assessed as in need of mental health intervention and the MDT agrees, appropriate care planning may include:

- Community based interventions and determining wait listing for an appointment.
- Immediate admission to an inpatient bed
- Acute response in the community.

Where the person's presentation is unclear, the decision regarding the next steps should be undertaken following a detailed medical assessment.

Refer to [SMHS Acute Response Mental Health Emergencies/Crises Policy](#).

6. Documentation

All contacts with individuals must be fully documented in the Psychiatric Service online Information System (webPSOLIS) and the patient's medical record using the mandatory State-wide Standardised Clinical Documentation forms:

- MH Triage
- RAMP
- Mental Health Assessment

Triage staff will ensure that all contact data is entered into Psychiatric Services Online Information System (PSOLIS/webPSOLIS).

Refer to the following Department of Health documents:

- Operational Directive [DOH Important Policy Information for PSOLIS Users](#)
- Mandatory Policy [State-wide Standardised Clinical Documentation \(SSCD\) for Mental Health Services](#)
- DoH [Triage to Discharge Mental Health Framework for State-wide Standardised Clinical Documentation](#)

7. Compliance monitoring and legislative penalties

7.1. Compliance monitoring

Each SMHS site or Service Director /Co-Director is to ensure compliance with this policy.

Compliance monitoring methods include:

- Regular clinical incident monitoring.

7.2. Legislative obligations

There are no legislative obligations relating to this policy for which a statutory penalty is imposed for a breach.

8. Legislation

- [Carers Recognition Act 2004](#), Schedule 1

9. Additional references

9.1. Department of Health policies

- [WA Health Consent to Treatment Policy](#)
- [MP 0099/18 Community Mental Health Status Assessments: Role of Mental Health Clinicians Policy](#)
- [Safety Planning for Mental Health Consumers Policy](#)

9.2. Other references

- [Chief Psychiatrist of Western Australia \(2015\). Chief Psychiatrist's Standards for Clinical Care as required under Section 547 of the Mental Health Act 2014. Perth, Chief Psychiatrist of Western Australia](#)
- [DOH Important Policy Information for PSOLIS Users](#)
- [MHA 2014 PSOLIS Business Operating Rules](#)

9.3. Document Control

Version	Published date	Effective from	Review date	Effective to*	Amendment (s)
GP 5	05/02/2024	02/02/2024	02/02/2027	2027	

10.4. Approval

Approval by	Executive Director Clinical Service Planning and Population Health, Kate Gatti
Approval date	02 February 2024

This document can be made available in alternative formats on request.

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